

1. Administrative & Study Identification

Full Study Title: A Phase I Dose Escalation and Expanded Cohort Study of PF-06821497 (Mevrometostat) in the Treatment of Adult Patients with Relapsed/Refractory Small Cell Lung Cancer (SCLC), Castration Resistant Prostate Cancer (CRPC) and Follicular Lymphoma (FL) **Short Title/Acronym:** A Study of Mevrometostat for Treatment of Relapsed/Refractory SCLC, Castration Resistant Prostate Cancer, and Follicular Lymphoma **Protocol Number:** C2321001 **NCT Number:** NCT03460977 **Posting ID:** 817732042169621517 **Sponsor Name:** Pfizer **Company:** Pfizer **Trial Status:** Recruiting **Phase of Development:** Phase 1 **Investigational Product:** Mevrometostat (PF-06821497) **Medical Monitor:** Pfizer Clinical Operations

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, maximum tolerated dose (MTD) based on Dose-Limiting Toxicities (DLTs), and preliminary efficacy of mevrometostat alone and in combination with standard of care (e.g., enzalutamide) in patients with advanced solid tumors and lymphomas. **Secondary Objectives:** To evaluate time-to-event anti-tumor activity including progression-free survival (PFS), objective response rate (ORR), duration of response (DoR), decline in prostate-specific antigen of $\geq 50\%$ from baseline (PSA50), time to first skeletal related event, time to symptomatic skeletal related event, overall survival, and pharmacokinetics (PK) including Maximum Observed Plasma Concentration (Cmax).

2.2 Background & Rationale To address the unmet medical need in relapsed/refractory SCLC, CRPC, and FL. In CRPC specifically, resistance to androgen receptor pathway inhibitors (ARPI; e.g., enzalutamide) is a significant challenge. Combining an ARPI with therapies that modulate alternative signaling pathways, such as epigenetic modifiers like the EZH2 inhibitor mevrometostat, is a promising treatment approach to delay or overcome antiandrogen resistance.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Enzalutamide alone in randomized dose-expansion cohorts) and Single-arm (Dose escalation cohorts) **Blinding:** Open-label **Randomization:** 1:1 (in dose expansion

cohorts comparing Mevrometostat + Enzalutamide vs. Enzalutamide alone) **Study Parts:** * **Part 1 & 2:** Closed for enrollment. Tested monotherapy (1A, 1B, 1C) and combination therapy (2A escalation, 2B/2C expansion). * **Part 3:** Open for enrollment for men with CRPC who progressed from their last treatment. Consists of a Bioequivalence (BE) substudy testing 2 formulations of mevrometostat (3 single doses over 3 periods), and a Drug-Drug Interaction (DDI) substudy to evaluate the effect of strong CYP3A4 inhibitors.

3.2 Planned Enrollment & Duration Target **\$N\$:** 433 participants
Number of Sites: Global, multi-center (e.g., United States, Spain, Poland, Japan, China, etc.) **Estimated Study Start:** 17-Apr-2018 **Estimated Primary Completion:** 20-Apr-2028

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria (Part 3 Key) 1. Histological or cytological diagnosis of castration resistant prostate cancer. 2. Eastern Cooperative Oncology Group (ECOG) Performance Status (PS) of 0-2 with expected life expectancy of at least 6 months. 3. Adequate bone marrow, renal, and liver function.

4.2 Exclusion Criteria 1. Prior irradiation to >25% of the bone marrow. 2. QTcF interval >480 msec at screening. 3. Hypertension that cannot be controlled by medications (>150/90 mmHg despite optimal medical therapy). 4. Known or suspected hypersensitivity to PF-06821497, any components, or enzalutamide. 5. Active inflammatory gastrointestinal disease, chronic diarrhea, known diverticular disease or previous gastric resection or lap band surgery. 6. Current use or anticipated need for food or drugs that are known strong and moderate CYP3A4/5 inducers or inhibitors. 7. Prior enzalutamide within the last 4 weeks. 8. **DDI Substudy Specific:** History of CHF or evidence of ventricular dysfunction, fructose intolerance, or coadministration of CYP3A4 substrates.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * **Part 3 Maintenance Phase:** Mevrometostat 2 times a day and enzalutamide 1 time a day by mouth until cancer is no longer responding. * **DDI Substudy:** Mevrometostat 2 times a day, with or without enzalutamide 1 time a day, and/or itraconazole 1 time a day based on a preset schedule. **Investigational Drug:** Mevrometostat (PF-06821497) **Standard of Care Drug:** Enzalutamide (Trade Name: Xtandi; ATC Code: L02BB04; EMA Reference: [Xtandi EPAR Product Information](#)) **Route of Administration:** Oral

5.2 Concomitant & Prohibited Therapy Permitted: Standard concomitant medications and ongoing Androgen Deprivation Therapy

(ADT) for CRPC cohorts. **Prohibited:** Current use or anticipated need for food or drugs that are known strong and moderate CYP3A4/5 inducers or inhibitors (except specific use of itraconazole in the DDI substudy).

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
1	-28 to 0	Informed Consent, Medical History, Tumor Imaging, ECG, Labs
2	0 to 21	Assessment (Part 3) Varying BE Substudy (3 single doses over 3 periods); DDI Substudy (preset schedule with/without itraconazole/enzalutamide)
3	21 to 42	Maintenance 21-Day Cycles Safety Labs, Efficacy/Radiographic Assessment, PK Sampling, Diary Review
4	42 to 63	End of Study Disease Progression Final Vital Signs, Exit Interview, IP Accountability, Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints

- * **Safety & MTD:** Percentage of patients with dose limiting toxicities (DLTs) to determine maximum tolerated dose. Overall safety profile including adverse events graded by NCI CTCAE version 4.03.
- * **Preliminary Efficacy:** Evaluated by disease-specific response criteria: RECIL for lymphoma, RECIST 1.1 for solid tumors (including SCLC), and PCWG3 for CRPC. Progression-free survival in Part 2B for patients with CRPC.

7.2 Secondary Endpoints

- * Time-to-event anti-tumor activity: Progression-free survival (PFS), Duration of Response (DoR), time to first skeletal related event, and time to symptomatic skeletal related event.
- * Decline in prostate-specific antigen $\geq 50\%$ from baseline (PSA50).
- * Overall survival (OS).
- * Pharmacokinetic Parameters: Maximum Observed Plasma Concentration (Cmax).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Monitored continuously and graded by NCI CTCAE version 4.03. **Serious Adverse Events (SAEs):** Standard reporting timelines, typically within 24 hours of site awareness. **Data Monitoring:** Managed by internal/external safety review committees for dose escalation decisions and DLT evaluations during the first cycle.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy; Safety Set for all TEAEs/SAEs. **Sample Size Justification:** Target enrollment of 433 is based on necessary

event milestones and statistical power across dose escalation, expansion cohorts, and DDI/BE substudies. **Handling of Missing Data:** Handled per standard institutional imputation rules for time-to-event oncology endpoints.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine onsite and remote monitoring visits for data verification. **Audit Trail:** Details on eCRF locking, query resolution, and data clarification forms.

11. Study Identifiers & Governance

Primary ID: C2321001 **Registry Number:** NCT03460977 **Posting ID:** 817732042169621517 **Sponsor/Lead Organization:** Pfizer **Preferred UMLS Name:** Malignant Neoplasms **Official Regulatory Title:** A Phase I Dose Escalation and Expanded Cohort Study of PF-06821497 (Mevrometostat) in the Treatment of Adult Patients with Relapsed/Refractory Small Cell Lung Cancer (SCLC), Castration Resistant Prostate Cancer (CRPC) and Follicular Lymphoma (FL)

12. Clinical Framework (Oncology Specific)

Disease Areas: Metastatic Castration Resistant Prostate Cancer (mCRPC), Small Cell Lung Cancer (SCLC), Follicular Lymphoma (FL)

Taxonomy & Disease Semantics: * **Lymphoma:** Preferred Name: Lymphoma; Semantic Type: Neoplastic Process; MeSH Code: D008223; SNOMED ID: 118600007. Definition: A cancer originating in lymphocytes and presenting as a solid tumor of lymphoid cells.

Diagnosis Threshold: Histological/cytological confirmation of disease progression. **Medical Methodology:** EZH2 Inhibitor (Mevrometostat) +/- Standard of Care (Enzalutamide) **Optimization Target:** Maximum Tolerated Dose (MTD) and anti-tumor efficacy.

13. Study Procedures & Methodology

Management Pathway: Standard Oncology Care Pathway **Education**

Protocol (HCP): Protocol training, substudy schedule management (BE and DDI parameters), & safety management. **Education Protocol**

(Patient): Dosing diary tracking, CYP3A4 dietary/medication restrictions, and AE reporting instructions. **Quality Audit**

Mechanism: Clinical trial monitoring and pill count/adherence monitoring.

14. Observation & Follow-Up Schedule

Total Duration: Follow-up until disease progression, death, or withdrawal
Onsite Visit Cadence: Coinciding with 21-day cycles and specific preset schedules during Part 3 Assessment Phase.
Remote Monitoring Frequency: Inter-cycle telehealth checks as required by protocol.
Checklist Review Interval: End of each cycle.

15. Sign-off & Approval

Role	Name	Signature	Date	_____	_____	_____	_____
Principal Investigator	[Insert Name]	_____	[DD-MMM-YYYY]	_____	_____	_____	_____
Clinical Operations Lead	[Insert Name]	_____	[DD-MMM-YYYY]	_____	_____	_____	_____
Lead Statistician	[Insert Name]	_____	[DD-MMM-YYYY]	_____	_____	_____	_____